

THE BRITISH JOURNAL OF DERMATOLOGY.

APRIL, 1905.

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By H. RADCLIFFE-CROCKER.

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The following description is drawn up from nine of the cases, all males, which, in the main features, closely resemble each other. The remaining case, a lady, had some important differences which will be discussed later.

So far, all the cases have been adults, though some of them were young. The lesions are evolved in patches of a pale pink or yellowish hue on the limbs and trunk, the uncovered parts, such as the face and hands, being free or very slightly affected. Generally, the patches come out very gradually and in small numbers and, in the main, symmetrically, but as the older patches never go away spontaneously, while fresh ones are continually evolving at short or long intervals, large areas are involved, and in the course of years (in one case, months) the whole trunk and limbs are crowded with lesions, though there are always spaces of normal skin intervening, or sometimes completely enclosed by the diseased process, where the original patches have coalesced. For the most part the original patches are discrete and enlarge but little after their formation, unless they

merge into adjoining patches, when hand-sized or larger areas may be formed.

With regard to individual patches, they are usually of oval or elongated form, arranged symmetrically in oblique lines on the back in the direction of the ribs, probably in the lines of fission, more or less horizontal in direction in front, and often, but not always, in vertical lines on the limbs. On the latter, especially the thighs, they not infrequently present the appearance of streaks formed by the finger, the upper part of the stroke being abrupt, and the lower shading off. This may sometimes also be seen on the trunk. The majority of the single patches range from one to three inches in their longest diameter; the borders are not very well-defined nor raised above the rest, but there is no difficulty in discerning the morbid from the healthy skin. They are not raised above the surface, but may be rather deep in the cutis. Infiltration can often be distinctly felt when the patch is pinched up in comparison with the adjoining healthy tissues, but in the more recent and smaller patches it is imperceptible, and occasionally they look like mere stains. Their colour is either pale pink or yellowish; in some cases the yellowish hue is pronounced, in others absent or nearly so; on the lower limbs the pink hue predominates. The surface is smooth on the trunk, but is often slightly rough on the arms and thighs, and below the knees maybe distinctly rough or even in branny scales. The patches are never so marked on the upper as on the lower limbs, the palms are always free, and the backs of the hands are generally unaffected, but sometimes there are a few small patches below the wrist. The face is nearly always free, though I have seen faint patches in one case. There is very little to suggest that the disease is inflammatory, and itching is quite absent in most of the cases; a few patients said they had some itching when hot, but only in one case was it really complained of, and that only in the early evolving stage of the patches. The initial site for the lesions varies; the thighs are the most frequently first affected, the legs next in frequency, and then the trunk. The lower limbs, too, are generally more crowded with lesions than other parts.

The duration of the disease may be very long. My first case had been developing for over ten years, others had been only for a few months; but in the case of a medical man, over 50 when I saw him, he said that patches first appeared on his legs when he was a house-surgeon, and had been

slowly evolving ever since, so that after thirty years he was pretty thickly covered, as none as far as he knew had gone entirely away, though they had temporarily disappeared when he had rubbed in chrysarobin ointment, but had gradually returned to their old site.

The disease is compatible with perfect health; and even when there was any departure from the normal there was no reason to suppose that the abnormality was in any way connected with the skin lesions, while the majority of the patients had above the average health for their age.

While there appears to be no tendency in the disease to spontaneous involution, they are not, as the case narratives show, altogether rebellious to treatment, and in at least two cases a cure appears to have been effected and in others some improvement, while in a residue no improvement could be noted. The agents which appeared to have a good effect are salicin in 15-grain doses at least three times a day, which by itself entirely cured a recent case (Case 5) of only two months' duration and of rather acute development, and vasogen iodine 10 per cent. rubbed in is a useful supplement and materially aided in the cure of Case 4. In some cases, salicin has failed to make any marked impression on the lesions, while in others the patient has not gone on with it sufficiently long to test its merits. As might be expected, it has been most successful when the disease has been present for a short time.

The only female case, a lady aged 47 years, resembled the other cases in its gradual evolution, long duration, absence of itching, in the persistence of the old patches with continual evolution of new ones, in its limitation to the covered parts, and in the general good health of the patient. The differences were in the patches being distinctly scaly all over the body; though the scales were small and even powdery in most parts of the body, they were, as usual, rather larger and more abundant on the legs. The patches were also more decidedly red than in the other cases; while there was some spontaneous improvement in the summer, in winter the patches cracked and smarted. This patient, who had been affected with the disease for ten years, had had the most varied drug and spa treatment, including cacodylate of soda injections for three months without any material effect; but after nine exposures to the Röntgen rays, the part exposed entirely cleared up, while the disease was unchanged on the inner side of the leg which the rays had

not reached. I intend, therefore, to make use of the rays wherever practicable.

As regards to *etiology*, it is chiefly negative. There is a large preponderance of males, and all the cases have been over 20 years old, while 56 years is the oldest I have met with. In no case could an exciting cause be made out; two of the patients had had syphilis, but it did not appear to have any etiological importance, and in one of them antisiphilitic treatment was tried vigorously for twelve months without effect.

CASE 1.—Mr. O——, draper, aged 30 years, was first seen by me on March 5th, 1902. The disease had been present ten years. From the commencement none of the patches had gone away. They appeared simultaneously inside the arms and thighs. They increased in numbers very slowly for a long time, and were confined to the limbs until three years ago, when they attacked the trunk, and during the last year have greatly increased in number; in fact, most of them have appeared in the last twelve months. His father was drowned and his mother died of fatty heart at the age of 51 years. When first seen by me the disease was in yellowish patches which commenced four inches above the nipples, but were not abundant till the line of the nipples, and they were less numerous below the umbilicus than above it. They were rather thickly arranged in horizontal elongated patches from 1 to 3 inches long, and 1/2 inch wide, as if streaked by the finger, pale, pink, or yellowish in tint, rather well-defined, but the edges were not sharp, and when the patch was pinched up a slight infiltration or thickening could be felt in the skin. The longer patches were formed by coalescence of some of the smaller ones. The surface was quite smooth. On the sides, the patches inclined slightly downwards and forwards, but they were practically horizontal in front. On the back they were sparse, and faintly developed in the interscapular region, and not nearly so numerous as in front; but on the lower half of the back and sides they were in the form of yellowish red stains, without elevation or roughness, and they were more numerous than on the upper part of the back. Forearms: The patches were on the inner side chiefly, more numerous on the right side than the left, and thickly arranged between the wrist and elbow. They were not elongated, but roundish, oval, or irregular, about three quarters of an inch in diameter, and somewhat brighter in tint than on the trunk. The surface was faintly rough, and on

pinching up a patch it was slightly thicker than usual. There were about eight to ten patches on the right upper arm, while the left was almost free, and there were not nearly so many patches on the left forearm as there were on the right. On the thighs: They were most abundant on the inner side, and many more on the right side than on the left. There were scarcely any patches on the front of the thigh, but there were a few on the outer side. The patches sloped downwards and inwards, were oval and broader than the trunk patches, but still like finger-streaks. The legs were much more densely covered with coalesced irregular patches, some enclosing healthy skin, and the surface was rougher than the lesions on the rest of the body. On the neck, there were a few ill-defined spots about one inch square in area. There was slight blotchy redness of a not very obvious kind on the face, scarcely perceptible on the forehead. There was some seborrhœa capitis, but not so much as formerly, as he uses brilliantine. The patches seldom itched unless he got very hot. No illness preceded the eruption; in fact, he has never had any illness; his tongue was clean, and he looked and has always been perfectly healthy. The general aspect was somewhat that of a general orbicular seborrhoic eczema, except that for the most part the surface was smooth.

He has consulted dermatologists and others, but nothing he has taken or used has done him any good. I only saw him once.

CASE 2.—Mr. H——, aged 37 years, manager of a factory, came to me on April 2nd, 1902. His general health was very good. The disease had been present five years, and began on the right fore-arm and a little later attacked the left. He has never been free since it first appeared, but thinks some patches have faded and others come out. In the last winter he had been decidedly worse, for the patches had certainly increased during the last few months. On the fore-arms, they were nearly symmetrical, and were quite so at an earlier stage. The lesions were yellowish or pale red patches; the simple ones were elongated, but the compound ones irregular in outline; they were from $\frac{3}{4}$ to $1\frac{1}{2}$ inches long. The surface, with a lens, could be seen to be very slightly roughened, but this was not perceptible to the finger—but when pinched up there was decided thickening, though they were not raised. There were no patches on the back of the arms or fore-arms and none on the trunk, but there were similar patches on the inside of the thighs

and on the legs, some of them larger than the arm-lesions but less distinctly patchy. There was a large, irregular area on the back of each thigh, and a few roundish ones scattered about the limbs. The face was quite free, there was no seborrhœa capitis and there was no itching or other sensory symptoms in the patches. He was one of ten brothers and sisters who were in good health, except one who died of syphilitic paralysis. His father died aged 71 years, and his mother, aged 63 years, was alive and well. He was given salicin, gr. xv *ter die*, and a lotion of glycerine of lead and liquor carbonis detergens. I saw him again on July 3rd, when the eruption had cleared up considerably, much of it having faded, leaving only a slight stain, but here and there it was still yellow and slightly rough. He was much better when he took the medicine regularly, but when he stopped it some of the eruption returned. The salicin was increased to 20 grains. I did not see him again until July 23rd, 1904. He then said he took the medicine for three months from July 3rd, 1902, and then stopped it. He had used the lotion a little longer. On the back of the legs there were large patches nearly covering the entire area, but not much in front. It was more distinctly red here than elsewhere, and there was very little thickening. There were only one or two patches on the left thigh, but a dozen or more on the right. The trunk still remained free. The initial patch was on the right wrist seven and a half years previously, but had disappeared. Some few fresh ones still developed occasionally. He was told to continue the salicin and rub on 10 per cent. vasogen iodine.

CASE 3.—Mr. H——, aged 56 years, butcher, came to me with skin lesions of which he had only been aware three weeks, but they may have been there longer as he had to get up so early to attend the markets that he rarely saw his own skin. He was a stout healthy-looking man, but he had syphilis in 1879 and had then suffered from iritis; he was under treatment for two years. He suffered from constipation and had done so since he had enteric fever as a young man. Urine had no albumen nor sugar. He came of a long-lived family, his father having died at 86 and his mother at 74. He had seven brothers and sisters alive out of thirteen. The lesions were few in number and situated symmetrically over the lower ribs on each side, the patches sloping obliquely downwards from the back to the front. One on each side was 6 or 7 inches long and compound. There was a single one much smaller (2-3 inches) symmetrically situated in a line with the large patch, and there

were three or four others above the major patch on the back. There was one distinct patch over the left scapula and about half a dozen fainter ones about the back; a few faint ones were present on the back of each arm and forearm. The lower limbs were free. The patches were erythematous in aspect, the larger ones brightish red, the others paler. The surface was very faintly roughened to the touch, and there was very distinct thickening when the skin was pinched up, but they were not raised above the surface. The borders of some of the patches were well defined, while others shaded off gradually.

He was given salicin internally and, to rub in, 10 per cent. vasogen iodine.

On February 11th there was decidedly less thickening on the left side but no noticeable difference on the right. He complained of indigestion, so the salicin was stopped and bicarbonate of soda given him, with tincture of nux vomica. On March 17th the thickening was much less and the patch on the left forearm was gone. On May 8th it had all cleared up and only left slight staining. As he has not come again he has probably remained well. The resolution of the patches was in this case probably, to be chiefly attributed to the vasogen iodine.

CASE 4.—Mr. M——, aged 54 years, a gentleman in good circumstances, was brought to me on June 2nd, 1903, by Dr. Lovell. All his family were long-lived. His grandfather was killed by an accident aged 93 years, but his father died of cardiac disease, aged 67 years. The disease the patient suffered from commenced early in April, *i.e.* two months before I saw him, on the right leg, and by June had extended all over the lower limbs and on the trunk, nearly all over the back, but there was scarcely any on the front. On the forearms, the patches were more on the extensor than the flexor surface. The eruption consisted of irregular, pale, erythematous patches from a square inch to the palm in size, with distinct infiltration in most of them. On the back, they were arranged symmetrically in the direction of the ribs, *i.e.* sloping downwards and forwards from back to front. In the early stage, on the back, they could be traced to commence as minute papules at the hair-follicles, and gradually the intervals between were filled up until infiltrations with a uniform surface were formed. Some itched, but none severely, and many did not itch at all.

Salicin 15 grains three times a day, with 5 minims of tincture of nuxvomica, was prescribed. On June 23rd I saw him again, and there was then distinctly less thickening on the thighs and legs and the eruption was somewhat less bright in some parts. The longitudinal patches over the scapula were still thickened, but there was less infiltration in the patches on the upper limbs and on the forearms they were yellower.

In response to my inquiry Dr. Lovell wrote me on November 29th, 1904, that he saw the patient on July 19th, 1903, and the eruption appeared to be gradually fading away. He next saw him for a sore throat in January, 1904, and the patient told him that the skin affection had gradually left him. He had had no other treatment than taking the salicin which I prescribed. This case is the most satisfactory and rapid in its involution under treatment, as it was also the most rapid in its evolution, and came earlier under my observation than any of the others.

CASE 5.—Dr. D——, aged 53 years. In 1876, while a house-surgeon, he first noticed a patch on one calf; since then the patches have gradually increased in number, and some of them in size. Chrysarobin kills them down for a time, but they re-form in the same place. In his family history a sister died of Addison's disease, and one daughter is slightly phthisical. His own health is good and he is well nourished.

When seen by me on February 4th, 1904, there was a large irregular lesion occupying nearly the whole of the interscapular region and numerous others on the back, abdomen, and thighs. They were of a dull pale lemon colour; some were distinctly thickened when the skin was pinched up; in others this was not recognisable. On the thighs, they were pinkish and slightly roughened. On the body they were smooth, elongated, oblong, oval, and well defined, symmetrically placed, and on the back in oblique lines sloping down and out from the spine in the lines of cleavage, *i.e.* corresponding to the rib slope. They seldom itched or gave any trouble.

CASE 6.—Mr. H——, aged 37 years, a draper. The disease had been present two years. When seen on April 26th, 1904, he had numerous patches on the calves and front of the legs, some three or four inches across, with distinct thickening. They were round, well-defined, and when pinched up were hard

as compared with the adjoining healthy skin. Besides these there were more recent superficial irregular patches symmetrically arranged on both thighs. In the interscapular region and in front under the breasts were slightly yellowish stains, but no other alteration of the skin could be seen or felt, the surface being quite smooth. There was occasionally some itching, but never severe. There was constipation present, but no other symptom of ill health.

He was ordered fifteen grains of salicin three times a day, and to rub in ten per cent. vasogen iodine. He was seen again on June 1st, when there was less thickening in some of the patches, but no other change.

CASE 7.—Mr. D——, aged 34 years, was seen first on November 3rd, 1904, in consultation with Dr. Payne. He had had a chancre in 1896. He had a severe sore throat and a rash, which only lasted a short time. He was treated with mercury for eighteen months.

The present eruption began from three to four years ago, and for the last twelve months he had been treated by Dr. Payne with the biniodide of mercury mixture, 1/16 grain three times a day, and also with mercurial inunctions, but without making any material effect upon the present lesions. When I saw him the trunk was only slightly affected. There were a few pale yellow patches below the nipples, elongated and nearly horizontal. On the back, there were one or two commencing yellowish spots in the interscapular region, but in the lumbar and sacral regions they were fairly numerous, and also yellowish and elongated horizontally. On the thighs, the patches were very abundant in vertical elongated streaks about half an inch wide, as if the finger had been wiped down the limb in two- or three-inch pale yellow streaks. On the legs, the patches were larger, irregular in outline from coalescence, and of a yellowish-pink colour. The eruption was very symmetrical, many of them, especially on the thighs, were slightly but distinctly thickened when pinched up. On the anus and forearms there were a good many patches, but not so many as on the thighs, where, arranged in the line of the limb, they were of a paler yellow colour. The patient said that they itched very decidedly almost always at night and very often in the day. Sometimes they were sore. They did not itch when they first came out. The eruption began on the thighs, and although fresh ones were coming out from time to time, none, so far as he knew, had faded or gone away.

He was ordered to rub in vasogen iodine and take salicin 15 grains three times a day. I heard about two months later by letter that there was no material change, but he had not used the iodine local application.

CASE 8.—Charles C——, aged 32 years, tailor, came to University College Hospital on October 4th, 1904. He stated that the skin lesions had commenced four years previously, appearing first on the thighs, then on the legs, and a little later on the forearms. He did not remember when the body was first affected. Fresh patches have appeared from time to time, but none have gone away. The lesions consisted of irregular patches from half to several inches in diameter symmetrically distributed over the trunk and limbs. On the back, they were in elongated or oval patches, symmetrically arranged in oblique lines corresponding to the direction of the ribs. They are well defined from the healthy skin, not raised above the normal skin, and the border was not raised above the central portion either to sight or touch. In front, there were large areas due to coalescence of several patches, but the smaller patches were elongated and horizontally placed. On the thighs near the groin the patches ran obliquely round the limb, but lower down were vertical. The general colour was yellowish pink, but in some the yellow, in others the pink predominated. When pinched up, many of the patches were distinctly thickened, but in the more recent this could not be felt, and some appeared as mere stains. The surface was smooth on the trunk, but on the lower limbs there was some roughness of the surface, more marked below the knee, but not actually scaly. The patches on the trunk were larger than on the limbs, and healthy areas of skin were sometimes completely enclosed by the diseased patches. The upper part of the chest, neck, and face were free, except a small patch on the lower lip. There were also large spaces of unaffected skin on the thighs, but only small ones on the upper limbs. On the back of the hands near the wrists, there were some small patches. The mucous membrane of the mouth was normal. There was no itching nor other subjective sensation. There was a history of phthisis in his mother and brother, but the patient was well nourished and healthy, except that he was liable to bronchitis, and had some enlarged post sterno-mastoid glands on the left side. He was put upon salicin gr. xv *ter die*, and vasogen iodine rubbed on the forearms, but so far no decisive result has been obtained. Mr. Pernet has obtained a piece of skin

from the right forearm, where the lesion was apparently superficial. His report is appended.

CASE 9.—A gentleman, aged 29 years, in whom the disease has been present for four and a half years, was seen with Mr. George Pernet, who showed him at the Dermatological Society of London in November, 1904, and the notes of his case were published in the December number of the *British Journal of Dermatology*, vol. xvi, p. 457.

There remains only the case of the lady, which requires separate consideration.

CASE 10.—Mrs. H—— was sent to me by Dr. Vassie on January 18th, 1902. She had suffered from the disease for nine or ten years, had seen other dermatologists, and visited sulphur and other spas. The disease had begun with a single patch on the arm, and after remaining single for a few months, had spread and extended almost all over the body and limbs.

When I saw her, the lesions were practically all over the trunk and limbs with ill-defined pale red patches with powdery roughness and decided thickening of the whole cutis of the patches, most marked in a 3-inch patch, situated high up on the right loin. The thigh patches were also thickened, but on the arms and forearms the thickening was less marked. On the front of the legs, the patches were irregular and more decidedly scaly, but not like either eczema or psoriasis. The eruption, as a whole, was like a seborrhoic eczema but less defined, and in parts the lesions were very pale and shaded off into the healthy skin. The neck, face, and hands were free. It smarted in cold weather, but did not itch.

The catamenia were regular, and her general health when seen was excellent, but a year previously she had had a fibroid removed after it had begun to be troublesome for a month from hæmorrhage. Before that the catamenia were twice a month. There was slight seborrhœa capitis. Salicin, 15 grains three times a day, was given. On July 17th she was seen again, and the eruption on the legs was somewhat paler, and that on the arms was much paler, but over the scapula the lesions were thickened but paler; but she attributed the improvement to the warmer weather during the last

month, as there was generally some improvement in the summer, while in the winter it fell back, cracked, and smarted. She was not seen again until May 12th, 1904, having meanwhile had other advice, but without any improvement, and none of the patches had gone away. At Wiesbaden she had subcutaneous injections of cacodylate of soda in the month while she was there, and once a fortnight for twelve weeks after, but without benefit. It was noted that the leg's were distinctly red and scaly. On the rest of the limbs and body the patches were red and scaly but without thickening, except on the forearms, the patches on the left being distinctly thickened. The patches were large, many being palm-sized and in the aggregate covered three fourths of the body, but the face and hands were quite free, and the neck nearly so. There was no itching. As every previous treatment had failed, the possibility of the disease developing into Mycosis fungoides being entertained, although itching was absent, it was resolved to try the effect of the Röntgen rays on a portion of one leg. Nine exposures of ten minutes each at a distance of eight inches were given, and a month after there was slight improvement where the rays had been used, but it was not very decided; but on the left arm where vasogen iodine had been rubbed in there was decided improvement, the lesions being pale, less scaly, and there was some clearance as compared with the right side, which had not been treated. I did not see her again until February 7th, 1905, and she then told me that the improvement where the X-rays had been applied had continued, and in a few weeks after I last saw her, in August, 1904, the disease had entirely disappeared from the outer side of the leg below the knee, and the skin when I examined it was quite white and smooth on the part exposed and had been so for four months, while the inner side of the leg where the rays had not reached remained unaltered. She had not continued the vasogen iodine to the arm, and it had become scaly again.

General Remarks on the Disease.

From the above description and the cases related in support of it, what inference may be drawn as to the nature of the disease and its relation to other skin affections? Frankly, at present I am quite unable to even conjecture its pathology, or to suggest, with one exception, any relation to other dermatoses. In my first cases I supposed that it was an early stage of *Mycosis fungoides*, though the absence or trifling character of the subjective symptoms did not lend this any support. I clung to this theory, however, *faute de mieux*, for a long time, but it received its *coup de grace* when I met with a case of over thirty years' duration without any such malign development.

At the Dermatological Society of London, where Mr. George Pernet and myself have shown three cases, among other suggestions, that of a possible seborrhoïde or *Urticaria pigmentosa* have been made. Against the former, the absence of scaliness in a large proportion of the patches, that the patches are in the skin, not raised above, and that itching is an exceptional feature, together with the unchanging character of the lesions, effectually bar the diagnosis of a seborrhoïde. Neither can I find anything beyond the yellowish tint frequently, but not always, present to support the idea of *Urticaria pigmentosa*. The absence of itching in nearly all the cases of *Urticaria factitia*, while the patches are level with the normal skin, and their unchanging character, are all strongly against such a diagnosis, to say nothing of the extreme rarity of adult *Urticaria pigmentosa* and the improbability of one man meeting with nine cases of it. The histology also negatives this.

I can therefore, at present, only regard it as a dermatosis *sui-generis*, of which the pathology must be left an open question, as Mr. George Pernet's report on the histology of the portion of skin removed from Case 8 does not throw much light upon the pathology. There remains only the affection described by Brocq, to which, certainly, there are closer resemblances than to any other affection.

Brocq^[1] divides the group which he calls parapsoriasis into three groups:

First variety (very closely related to psoriasis), Parapsoriasis guttata. Jadassohn's case is probably to be referred to it.

Second variety (intermediate between Lichen and psoriasis), Parapsoriasis lichenoides, including Parakeratosis variegata of Unna, and Lichen variegatus (Crocker).

Third variety (closely allied to Seborrhœa psoriasiformis), Parapsoriasis in patches, corresponding to Erythrodermie pityriasique en plaques disséminées of Brocq, and of which cases have also been reported by J. C. White and C. J. White.

It is only with the third variety that comparison need be made, the deep colour and very small pattern, like a mosaic of the first two, sharply contrasting with the broad effects as of colour dashed on, in Xanthoerythrodermia perstans.

For the whole group Brocq gives the following characteristics:

- (1) An almost complete absence of pruritus.
- (2) A very slow evolution.
- (3) A distribution in circumscribed, sharply defined patches, whose dimensions are from 2 cm. to 6 cm. in diameter, and which are scattered here and there over the integument.
- (4) An almost complete absence of infiltration of the derma.
- (5) A pale redness (pinkish coloured).
- (6) A fine pityriasic desquamation.
- (7) An extraordinary resistance to the local applications usually employed in the treatment of psoriasiform or pityriasic seborrhœa, in fact, only yielding slowly and imperfectly to the most energetic application of pyrogalllic acid.

The special features of the third variety he describes as:

- (1) Being in patches, circumscribed, sharply defined from 2 cm. to 6 cm. in diameter.
- (2) They are scattered irregularly over the skin without any apparent system.

(3) The colour varies from a pale red to a brownish or livid red, according to the part affected.

(4) There is always present a fine pityriasic desquamation more or less marked in different cases.

(5) There are at times in some of these cases, aggregations of small flattened papules which may be considered as links connecting it with the second variety.

(6) There is no infiltration of the integument appreciable to the eye or touch.

(7) The face is rarely affected, and there is the same extremely slow evolution, great resistance to local treatment, and few or no subjective symptoms.

Referring to the whole group, he says: "We know nothing very definite about the etiology or pathology of these affections. They may appear at any age, but seem more frequent in youths or adults. He has observed cases in men and women and in all classes of society. They seem to be slightly more frequent in women than in men."

It must be confessed that there are many points of resemblance of this third variety to Xantho-erythrodermia perstans, and, as regards the lady, Case 10, it is probably what Brocq has described as "Erythrodermie pityriasique en plaques disséminées," although I should say there was distinct infiltration in a large proportion of the lesions, and many of the patches far exceeded the limits in size that Brocq lays down.

With regard to the other nine cases, the differences are somewhat more marked, and they should, at least for the present, be either kept apart or treated as a distinct variety.

The differences are:

(1) The patches are frequently much larger, 3 and 4 inches or more in their long diameter, and the margin is not very sharply defined.

(2) A distinct arrangement in lines in direction varying with the topography is observable in most cases.

(3) The colour is either pale red or distinctly yellowish.

(4) Instead of a fine pityriasic desquamation being always present, this is only distinct on the legs, sometimes just recognisable on the thighs and arms, while on the trunk it is absent, the surface being usually quite smooth.

(5) The presence of papules I have not observed, except at the commencement of Case 4.

(6) There is distinct infiltration in a large proportion of the patches, perceptible to the touch, though not to the eye.

The resemblances are: the absence of conspicuous pruritus, and there is often none; the occurrence in patches; the slow evolution; a pale redness; in many cases an extraordinary resistance to treatment. These are not enough to establish identity, but I confess that in my opinion they show greater resemblance to the affection I am describing than they do to psoriasis, and certainly my nine male cases would never suggest to any one a resemblance to psoriasis, and I should strongly demur to class them under parapsoriasis as a covering term.

Histological Note by George Pernet.

A piece of skin was removed from the extensor surface of the right forearm of the male case, aged 32 years (a Jewish tailor), where the more recent patches had appeared. Clinically there was practically no infiltration to be felt in that situation. The specimen was hardened in alcohol, cut in celloidin, and stained in various ways. The microscopical appearances were as follows:

Epidermis.—The stratum granulosum was either atrophied or absent. The stratum lucidum was absent except for traces here and there. The epidermis generally showed a slight amount of œdema.

Corium.—The vessels were dilated, with some cellular infiltration about them. The collagen appeared to be normal, with the exception of slight œdema, but the elastin was apparently reduced in quantity, especially in the papillary layer, and it was to some extent fragmented in places. It should be mentioned that the specimen was stretched on a small piece of cork in the process of hardening. The elastin stained, however, much as in the normal condition.

Altogether there was little to be gathered from the histology *quâ* cause, except that the appearances of the blood-vessels perhaps pointed to a general blood condition.

Dr. Thiele, Pathologist to University College Hospital, kindly examined the blood and reported: Total red corpuscles per c.mm., 6,560,000; total whites per c.mm., 21,878. Hæmoglobin, 84%. C.T., 65. Differential count of whites: Small lymphocytes, 18·2%; large lymphocytes, 9·4%; neutrophiles, 60·7%; oxyphiles, 1·6%; hyaline cells, 2·1%.

IDIOPATHIC MULTIPLE PIGMENT SARCOMA (IDIOPATHIC MULTIPLE HÆMORRHAGIC SARCOMA) OF KAPOSÍ.

By F. PARKES WEBER, M.D., F.R.C.P., AND PAUL DASER, M.D.

The patient, Jakob Z——, is a Polish Jew, aged 46 years, from Galicia, who has lived nine years in England, and says he has enjoyed good health. There is no evidence of previous alcoholism or syphilis. The patient was brought by Dr. Weber before the Dermatological Society of London on February 8th, 1905.

The present illness commenced about three years before, when he chanced to wound the sole of his right foot with a nail. A pedunculated growth, about the size of a cherry, arose from the wound. This description reminds one of the strawberry-like granulation tumours sometimes growing from small wounds or ulcers on the fingers, which have been termed “botryomycosis,” or rather “botryomycomata,” because at one time they were supposed to be caused by the “botryomyces.”^[2] The growth was removed by a doctor. Afterwards small bluish nodules developed from time to time on the feet and legs. Some of these nodules apparently undergo the following series of changes: They slowly increase in size, and after about three months constitute little pendulous tumours, which ultimately fall off spontaneously or else are knocked off or pulled off by chance; in this way local healing occurs, but fresh nodules form elsewhere. Though, as stated, some of the nodules become pedunculated growths, others appear to atrophy and merely leave brownish pigment in the skin, whilst others seem to undergo very little change, and, at all events, persist for a long time in their primitive form, namely, as hard bluish lumps under the epidermis. Treatment has been as yet without result.

Present condition (January, 1905).—On *the hands* about the knuckles and backs of the fingers are a few minute slightly elevated bluish nodules. On

the sole of the *right foot*, especially on the inner part, there are nodules in various stages of development. The minute ones, those in an early stage of development, are slightly elevated and of a bluish colour, similar to those on the hands. Others, further advanced in development, are small, sessile, or pedunculated hard outgrowths, without bluish colour and sometimes covered by thickened epidermis. There is a large patch of pigmented (brown) skin on the dorsal surface of the right foot, and in the neighbourhood of this pigmented area are some minute nodules of similar character to those already described. It is possible that the pigmented skin has been the site of minute growths which have undergone spontaneous involution, and have, as frequently happens in Lichen planus, left pigmentation of the skin behind them. The right thigh and leg are not affected.

The *left lower extremity* is the part most affected by the disease, and there is chronic, rather firm, œdema of the left foot and ankle. The nodules on the left foot are similar to those on the right, but are more widely distributed, and the left foot differs from the right in the absence of the pigmented area and in the presence of the œdema to which we have just alluded. Of the little sessile or pedunculated tumours on the back of the left foot and in the neighbourhood of the ankle-joint (see Fig. 1) the biggest are of the size of a large pea. There are likewise little tumours of various sizes—that is, in various stages of development—on the inner aspect of the left popliteal region (see Fig. 2) and in the neighbourhood of the patella, the smaller ones being bluish and slightly elevated, the larger ones being reddish, lentil-sized, sessile outgrowths.

There are no nodules or growths on other parts of the limbs, trunk, or head, with the exception of four or five minute purplish or bluish papules on the *glans penis* and neighbouring skin.



Figure 1

There is no evidence of any disease of the thoracic or abdominal viscera. The patient is not strongly built, but looks well nourished. He complains of various pains, but whether these are related to the disease or not is doubtful.

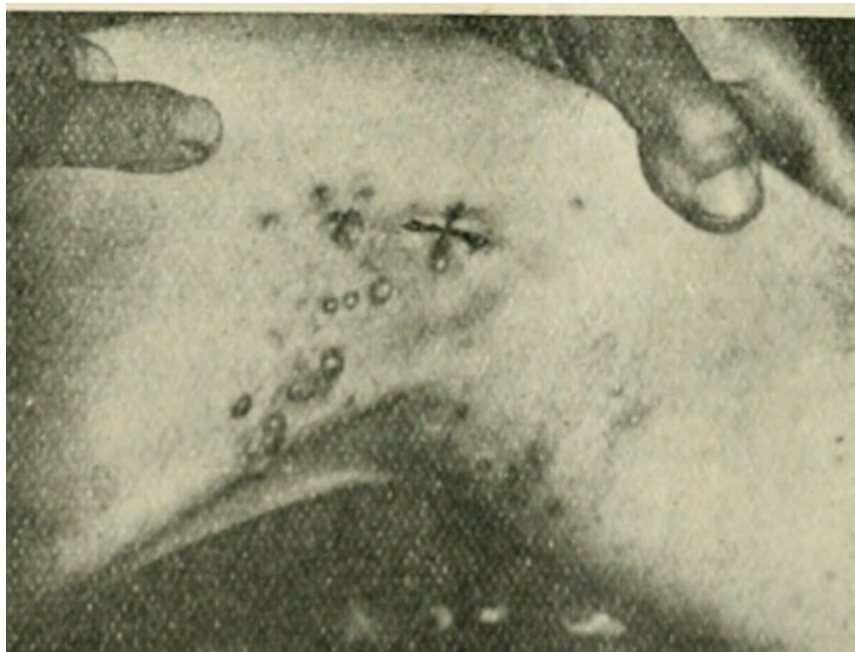


Figure 2

Microscopic examination.—One of the growths, of the size of a large pea, was removed from the left ankle, and a smaller one (lentil-sized) from the left knee. Sections of both were stained with hæmatoxylin and eosin-hæmatoxylin. The growths are seen to be situated in the corium, and are circumscribed. The epidermis over one of them is thickened. They consist of oval and spindle cells, which, in some parts, are more or less grouped into strands or bundles, and are cut in various directions. In some parts of the sections there are empty spaces (? lymph spaces) and in some parts there is homogeneous substance between the cells; in other parts there is extravasated blood. The cells in the larger and older of the two nodules are decidedly more elongated and fibrous-looking than in the smaller and more recently developed one. Dr. J. M. H. MacLeod is kindly going to make a further report of the microscopical features.

Remarks.—It seems to us clear that the case is a typical but relatively early one of the so-called idiopathic multiple pigment sarcoma (idiopathic multiple hæmorrhagic sarcoma) of Kaposi and later writers. For the literature on the subject we refer to the English summaries of Dr. J. H. Sequeira^[3] and Dr. Radcliffe-Crocker.^[4] The patient in our case is a male, as in nearly all other recorded cases of this disease; he is of the favourite age (between 40 and 50), and it may likewise be noted that he is a Galician Jew, as the patient of Sir Stephen Mackenzie^[5] and Dr. J. J. Pringle was. The minute bluish nodules in the corium may be regarded as the primary feature of the disease. They either remain for a long time without undergoing much obvious change, or they undergo spontaneous involution and completely atrophy, leaving behind them only a little brownish cutaneous pigmentation; or else (thirdly) they may increase in size, become much elevated, and form definite sessile tumours, which, later on, become pedunculated, and finally drop off. This is the cycle in regard to the development of the little pendulous tumours which constitutes one of the most characteristic features of the disease. Another characteristic feature of the disease, probably sooner or later present in every case, is the œdema which occurs in the parts most affected, especially in the lower extremities. It is persistent œdema, and results in an elephantiasis-like condition. It is well marked in the left foot and ankle of our case. The striking blue or purple colour of the small nodules (which is absent in the pendulous tumours) seems, as in other cases, to be due to the venous blood contained in the blood-vessels or extravasated. What pigment there is in the growths is probably derived from

blood. In our patient there is no evidence of gout as in certain cases described by Mr. Hutchinson.^[6]

The prognosis in the present case is not altogether unfavourable, considering that the patient's general health is good, that the disease is probably seldom of itself fatal, and that in some cases improvement or spontaneous cure has, apparently, occurred after the disease has lasted twenty years or more.

SOCIETY INTELLIGENCE.
THE DERMATOLOGICAL SOCIETY OF LONDON.

An Ordinary Meeting of the above Society was held on March 8th, 1905, at 5.15 p.m., Dr. J. H. STOWERS in the chair.

The following cases were demonstrated:

Dr. JAMES GALLOWAY showed the case of a young lady presenting an eruption on the left arm of unusual character. The lesion in some respects resembled those of granulomatous origin, but no definite diagnosis was given. This case will be subsequently noted in full.

Dr. T. J. P. HARTIGAN (introduced) showed a case of *atrophy of the nails*, a description of which will be found on page 147.

Dr. GRAHAM LITTLE showed: (1) A case of a *corymbose syphilide* in a man aged 44 years, with a history suggestive of a double infection with syphilis. The patient had been in the Army and had contracted syphilis in India, twelve years ago. He then had a chancre on the top of the glans penis, the scar of which was still visible. He was taken into the Military Hospital and treated for exactly ninety-seven days, his treatment ceasing entirely after this period. He had no secondary eruption and no further symptoms. Eight years later he had another chancre, also on the glans penis, but near the frænum. This was, according to his description, a deep ulcer, which was seven weeks in healing. He had no bubo following and no secondary eruption. Ten weeks ago he began to have the present eruption, which was the only general rash he had ever had. With the exception of the treatment during the actual presence of the two chancres, he had had no specific treatment whatever.

The eruption as seen on exhibition consisted of numerous groups of fairly large papules arranged in a somewhat herpetiform manner, with here and there a central larger papule surrounded by smaller ones, in the manner described as corymbose, but for the most part the groups were composed of papules equal in size; these groups were scattered over the back especially, the chest, the arms, the thighs, and the face, with individual large papules here and there upon the arms and legs. The mucous membranes were not affected.

The history seemed to point to a double infection, the interval between the two chancres being eight years.

(2) A case of *dermatitis artefacta* in a young lady, a private patient, who gave the following history. She was bitten by an old collie dog twenty-three days ago on the calf, through her dress. She was not seen by a doctor until a week or more later, and when seen by him she had a circular "patch upon the calf of vesicating erythema, surrounding the bite, with a vivid erythematous non-vesicating ring, about half an inch broad, surrounding the central patch, and separated from it by an intervening band of healthy skin, about a quarter of an inch broad." Four days later an exactly similar condition was noted on the lower part and inner aspect of the same leg, "a patch of vesicating erythema the size of a florin, surrounded immediately by healthy skin and then a ring of vivid erythema half an inch wide." The patient did not appear to be in any way neurotic. There was no apparent animus against the owners of the dog, who were old and intimate friends, and no breach of relations between them had taken place. She led a healthy active life and was, in fact, in robust health at the time. The ointment used in the first instance was obtained from a chemist; her doctor had prescribed carbolic ointment. The method of production was not ascertained and no adequate motive could be assigned.

(3) A case which was shown as *syringomyelia* with trophic ulcers on the upper arm and shoulder in a woman who had been under Mr. Ernest Lane's care for about twelve years, and had had numerous operations performed for a continually ascending necrosis of bone accompanied by trophic ulcers on the skin.

Some divergences of opinion were expressed as to the diagnosis, and this being at any rate a rare and interesting case, a more detailed report

will be subsequently submitted for publication in this journal.

Dr. J. M. H. MACLEOD showed a case of *grouped comedones* associated with acneiform lesions on the chest of a boy aged 2 years. The comedones were present chiefly on the sternal region, but several isolated groups occurred around the nipple and umbilicus. Only a few of the comedones had become inflamed or transformed into acne pustules. The mother had noticed the comedones about a month before exhibition. There was a definite history of local irritation in the case, for since the child was a few months old he had worn a piece of flannel over the chest in the affected region, and as the mother believed that he had a delicate chest the flannel had been frequently saturated with camphorated oil. The exhibitor intended to make a bacterial examination of the lesions with the object of trying to find the acne bacillus, and hoped to report the result at a subsequent meeting of the Society. Another point of interest in connection with the case was the fact that it occurred in a boy, as the large majority of reported cases have been in boys. Dr. MacLeod referred also to another case in an infant boy of about a year old, which he had seen on the afternoon of the meeting, but unfortunately had been unable to bring up, in which the comedones were grouped on the forehead and cheeks and in which there was no history of local irritation to account for the lesions.

Mr. MALCOLM MORRIS showed a case of early *Parakeratosis variegata*, a full report of which will be published in a future issue of the journal.

Dr. ORMEROD showed: (1) A case of *Lupus erythematosus telangiectodes*. The patient was a woman, aged 44 years, who stated that she had suffered from an eruption on the cheeks for the last eighteen months. On exhibition, there was a large red area situated symmetrically on either cheek, the redness being due to dilatation of the small vessels. Within these areas there were islets of paler and shiny skin, which showed superficial atrophy. A small area in each patch was covered with a crust which was difficult to remove, and when detached showed little tags and prominences on the under surface. The appearance of the eruption had not materially altered since the patient was first seen fourteen months ago.

All the members present agreed with the diagnosis of the exhibitor.

(2) A woman, aged 38 years, suffering from a curious *eruption associated with obscure nerve symptoms*. The history showed that four years ago she had suffered from some chest affection, accompanied by the expectoration of blood and offensive matter. She was ill three months, and during her convalescence she scratched her right thumb, and a lymphangitis of the arm, with axillary abscess, resulted. She next developed a painful swelling of the right elbow, which was opened and drained, and she thought a piece of bone came away; the arm was put up in splints. The right elbow had been stiff since that time, and during its healing she had developed pricking sensations in the fingers, which became contracted. There was then an interval of nine months' good health, after which, two years ago, she developed an abscess in front of the right thigh, which eventually broke, and laid her up for four months. Subsequently, about eighteen months ago, the rash appeared, at first round the site of the abscess, then spreading towards the buttock and loin, and finally extending down the leg.

On exhibition, the patient showed an extensive anæsthetic area over the right side extending up into the axilla, curving down both in front and behind at the level of about the sixth rib, and embracing the whole of the right arm and leg, with the exception of small islands on the palm, sole, and gluteal region, where sensation still persisted. The rash followed a similar distribution, but did not affect the arm, and tailed off on the leg with the appearance of isolated spots. The rash developed, according to the patient's history, in the form of red spots, on which a blister full of clear fluid formed. The fluid then became mattery, and an ulcer resulted. The development of an ulcer was heralded by a pricking sensation. On exhibition, the striking characteristics were the presence of patches of excoriated skin and white scars, surrounded by pigmentation. The patches were oval in shape, with the long axis running down the limb or transversely on the trunk. The more recent lesions consisted of red, raised papules with an excoriation on the summit. In spite of bandaging with boric acid ointment in the hospital a few lesions formed beneath the dressings, though more appeared outside. Examination of the supposed affected joints showed nothing abnormal; the reactions of the atrophied muscles to electricity were also normal. The anæsthesia was shown to be of psychical nature, and the exhibitor therefore considered most, if not all, of her

disabilities to be of an hysterical nature. He asked the opinion of the Society as to the nature of the eruption.

Most members, including the exhibitor, considered that the eruption was artificially produced by the patient.

Dr. J. J. PRINGLE Showed a case of *Lupus erythematosus associated with Raynaud's disease* in a highly neurotic woman, aged 44 years. The symptoms of Raynaud's disease apparently first manifested themselves in 1895, but were first recognised as such in 1896. Associated with the typical phenomena of recurrent local syncope and asphyxia of the extremities, more especially of the hands, there were attacks of severe griping abdominal pain. The exact nature of these attacks had never been accurately determined, but there were evidences of some chronic intestinal obstruction, which could not, however, be located. The exhibitor had witnessed several occurrences of typical Raynaud symptoms in the hands, and the nutrition of all the fingers was impaired, their tips being stumpy and atrophied, but no gangrene had ensued. The first manifestations of Lupus erythematosus showed themselves in 1900, in the temporal regions and scalp. They were regarded as "gouty psoriasis," and were treated by a physician professing the cult of homœopathy by repeated painting with pure oil of cade (!), under which they progressed rapidly, but in the wrong direction. She was also advised to winter in a high, cold, and dry locality, but she suffered terribly from this experience, which was carried out last winter in Switzerland.

The Lupus erythematosus was of severe inflammatory type and occupied a great portion of the scalp, denuding it of hair, and giving rise to a large atrophic scar which was the seat of peculiarly obstinate recurrent attacks of suppuration, probably referable to a dirty toupet, worn for cosmetic purposes. Symmetrical patches were also present behind the ears, inside the pinnae, and in the zygomatic regions, while there were present over the whole face a large number of telangiectatic spots averaging nearly the size of a pea. The nasal and buccal mucous membranes were healthy. The urine contained neither albumen nor blood-pigment, nor was there any history pointing towards hæmoglobinuria. The exhibitor asked the experience of members as to the value of treatment of such a condition by Finsen's

method, X-rays, and especially by high-frequency currents, which he was inclined to try as of possible value in both the associated conditions present.

Dr. Pringle's suggestion, that treatment by high-frequency currents afforded the best prospect of benefiting the patient, especially with regard to the Raynaud phenomena, was generally accepted.

Dr. DORE was of opinion that the disease would thereby be considerably ameliorated and possibly even arrested, but the attitude of the majority of members present was somewhat sceptical.

Dr. H. RADCLIFFE-CROCKER showed a case of *tuberculous gummata* (or gummatous tuberculides). The patient was a well-grown and well-nourished, healthy-looking girl, aged 18 years, who first came under observation on February 17th, when the following notes were made: The disease began at the age of 7 years, when a crop of lesions made their appearance. One of these occurred on the abdomen, near the right ilium, where a large scar, about 3 inches (over 7 centimetres) in diameter, was present. This condition lasted for a few months. Then very few lesions appeared up to the age of 15, when they became more numerous and had gone on ever since without any interval of freedom. For three years the patient had been subject to flat circumscribed infiltrations of the skin varying from a shilling to a crown in size. These lesions all broke down more or less into superficial ulcers, which healed in from two to three weeks. On the legs and thighs up to the crest of the ilium there were a large number of more or less circular scars varying in size, but most of them about as large as half a crown (3 centimetres). On the thighs, however, some were over 2 inches in diameter (about 5 centimetres). On the right thigh near the knee there was a recent lesion (two weeks old), which was about 2-1/2 inches by 1 inch (6 centimetres by 2 centimetres), dull red, with great thickening; it presented on its surface a pea-sized ulceration, which had formed within twenty-four hours of the time when the patient was first seen. A similar lesion, with a more advanced ulceration, on the right calf, had been present three weeks. Below the left knee there was a flat infiltration, slightly raised and well defined, about the size of a shilling (about 2 centimetres). On the right temple there was a small circular scar of 1/4 inch, which had resulted from a similar lesion to those already

described. The patient's lungs were unaffected, but the history was strongly tuberculous on the father's side, a large number of his relations having died of consumption, whilst some others were dying of the same disease at the present moment. The patient had lost one sister, aged 21 years, of phthisis, and a brother and another sister of "consumption of the bowels."

Dr. SEQUEIRA showed a case of *Lupus vulgaris* to illustrate the value of Dr. A. E. Wright's recent work upon opsonins. The patient, a Jewess, aged 20 years, was first seen at the London Hospital in the summer of 1900. The whole of the face and part of the neck were then much infiltrated and in parts ulcerated. The feet also were affected, parts of several toes having been destroyed. Owing probably to the fact that she was unable to walk, the patient was not seen again until February, 1905. Fortunately, photographs of the condition in 1900 were preserved, and these showed that the disease had extended very little in the four years and a half, although the patient had had no treatment whatever in the interval. The general health had improved, but the lupoid infiltration of the face and neck were well marked and there was still considerable ulceration, the ulcers being covered with dirty scab. The destructive ulceration of the toes had slightly increased. There were no physical signs pointing to tuberculosis of the internal organs. The glands were slightly enlarged, but this was not a marked feature.

Dr. Sequeira took the patient into the hospital, intending to use tuberculin. Before doing this, the blood was examined by Dr. Wm. Bulloch, who found that the opsonic index was 1·3, normal blood being 1. A high opsonic index, it may be mentioned, has been found by Dr. Bulloch in a certain proportion of the cases of Lupus attending the London Hospital, and particularly in the chronic cases doing well under the light treatment. As the opsonic index was already high in this case, it was deemed unnecessary to raise it by the injection of tuberculin, and fomentations were applied to the affected areas, to bring the opsonins to the diseased parts. The result was very striking. In a few days the lesions were paler, flatter, and clean. When shown at the meeting, after a fortnight's treatment with fomentations, the infiltration had markedly diminished, the raised margin being flatter and paler, and the ulcerated areas healed up, except upon the stumps of the toes.

The fomentations were of boracic lint soaked in hot water and covered with oil silk frequently changed. It may be mentioned that the improvement was

much more rapid than had been seen in similarly extensive cases treated by fomentations, the only known difference being the high opsonic index. The case is of importance as supporting Wright's work, and also as showing the value of examining the blood before injecting tuberculin. It also explains why some apparently severe cases do well under simple treatment.

Dr. GRAHAM LITTLE was able to confirm Dr. Sequeira's observations in this case from one at St. Mary's Hospital, in which a similar improvement had been noticed with fomentations in a patient whose opsonic index was as high as 2.

Mr. GERALD SICHEL (introduced) showed, on behalf of Sir Cooper Perry: (1) The boy, aged 8 years, who was shown at the February meeting as a case for diagnosis, and in whose case the chronic nodular annular patches had gradually increased in size; fresh, hard, painless, and but slightly tender, shotty nodules had also developed in the periosteum of the left temporal region. Dr. Galloway recognised the case as identical with one he had described in this journal as *Lichen annularis*. Dr. Pringle also had seen similar cases.

(2) A boy, aged 13 years, presenting symmetrical, painless *callosities*, covering in size an area of between a shilling and a two-shilling piece, on the back of each heel, and which he had noticed for the past eight or nine months.

THE DERMATOLOGICAL SOCIETY OF GREAT BRITAIN AND IRELAND.

A meeting of this Society was held on Wednesday, February 22nd, 1905, Dr. H. WALDO in the chair.

The following cases were exhibited:

Dr. W. CLEMENT DANIEL exhibited a man, a photographer by occupation, with a *symmetrical eruption upon the legs* resembling a chronic eczema, the irritation of which was very severe at times.

The general consensus of opinion was that this was a case of Lichen planus.

Mr. T. J. P. HARTIGAN showed (1) a case of *tertiary syphilis* in a man, aged 90 years, with a history of ten years' duration. It was recorded by Mr. Waren Tay, to whom the exhibitor was indebted for the case, that "there is a very warty patch on the tip of the elbow, the papillæ and epidermis being enormously hypertrophied. Adjacent is a horse-shoe patch, suggesting tertiary syphilis. There are no other patches, and only doubtful evidence of slight scarring, and of its serpiginous character. It is stated to have followed a blow which caused an abrasion, and that, working with his sleeves up, he often knocked the elbow." A histological examination showed a granulomatous infiltration of the corium very suggestive of syphilis, and there had been marked improvement under potassium iodide. The case was interesting as showing (a) that extreme old age was no bar to the occurrence of gummata, and (b) the enormous hypertrophy that may accompany the condition.

(2) A case of *atrophy of the nails following measles* in a girl aged 18 years. Three months after the exanthem all the nails were shed, and subsequently regenerated, except in the case of the thumbs, both index and middle fingers and the big toes, where they were either very thin, striated, and longitudinally fissured, or apparently absent, the posterior nail-fold having grown forward and become adherent to the nail-bed, through which more or less of the root of the nail could still be felt. No member of her family had any affection of the nails. There was no evidence of syphilis, and she had never had any other illness. The condition had persisted for thirteen years.

Dr. R. L. BOWLES remarked that anything which lowered the vitality of the system might produce various abnormalities of the nails, and he quoted the observations of Sir Samuel Wilks who had seen atrophy of the nails after scarlet fever.

(3) A case of *ulcer of the cheek* in a man, aged 52 years, of four months' duration. It was roughly circular in shape, at least 1 cm. in diameter, and scabbed over. The glands were not enlarged, and there was no evidence of syphilis. If it should turn out to be a rodent ulcer, he thought it was unusually rapid in its growth. He proposed removing it entirely, and he would report on it histologically at a subsequent meeting.

(4) A case of *acute Lupus erythematosus* in a married woman, with a history of five months' duration. It began as two small patches on the cheek in front of the right ear and extended to the face, the pinna and concha of the ear, and the neck. Soon after, the fore-arms and backs of the hands and fingers became affected. The scalp was very scurfy, and at one time she was much exposed to the sun. There was scaling, but no vesication or evidence of atrophy. There was a good deal of pain and tenderness, and she was quite unfitted for work.

(5) A case of *congenital pigmented naevus* in a girl aged eleven years. There were numerous small, dusky, pigmented patches and spots scattered upon the left side of the neck, shoulder, and breast. There were also a few spots on the lobule of the corresponding ear. They were said to be getting darker, and some were becoming papillomatous. They were thickly grouped around the nipple, the areola of which was deeply pigmented. There was also a patch of dark hair behind the ear on the same side.

(6) A case of *acquired pigmented nævus* in a boy aged 11 years. Two years ago he noticed a freckle beneath the left eye which gradually became darker, and it was now black. Four more soon followed immediately below the first, and then another appeared on the lower lip on the right side. Other spots were to be found on the trunk and neck.

Among the suggestions for treatment put forward by the members of the Society excision of each nævoid spot was mentioned as being the most efficient method of radical cure.

(7) A case of *Lichen pigmentosus verrucosus* in a girl aged 12 years. The condition started five years ago on the forehead, and extended to the neck, trunk, groins, and the front and inner aspects of the thighs. There were innumerable slightly pigmented, warty lesions, irregular in outline, and varying in size from that of a pin's head to that of a split pea. They were composed of a soft, friable substance that could be easily picked off with the finger-nail. He was not aware of ever having seen or heard of a similar case. A portion had been removed for histological examination, and he would endeavour to show a section before the Society on a future occasion.

This case excited considerable interest, but the majority of the members preferred to reserve their diagnosis until after the microscopic examination.

Mr. SPENCER HURLBUTT showed a *case for diagnosis*. The patient was a delicate-looking woman, aged 22 years, with a symmetrical eruption affecting both legs. The disease first appeared three months ago as a brownish-coloured spot on the outer side of the right calf, which was followed at intervals by similar ones on each leg, and these had gradually enlarged until the present time. There were now ten to twelve patches of varying sizes up to that of a florin situated upon the back and outer surface of the legs. The lesions consisted of fairly well-defined, non-indurated, circular, dull-red patches, with thin adherent scales, their general appearance being suggestive of psoriasis which had undergone treatment. The usual situations affected by that disease were, however, free from the eruption, and severe itching, especially toward night-time, was a prominent symptom.

The general opinion was that this was a case of seborrhœic dermatitis of a psoriasiform type.

Dr. GRAHAM LITTLE showed (1) a case of *Tinea cruris* in a private patient, a Japanese gentleman, an exponent of the art of ju-jitsu. The patient had noticed the eruption for about a month and a half, but could not be certain that he had not the first trace of it before he left Japan, six months ago. He was engaged in teaching the national form of wrestling, and in his exercises had to strip almost entirely, so that he might well have contracted the disease from a pupil. There were now large patches of scaly dermatitis occupying the perineum and groins with extensions on to the scrotum. On the left side of the cheek there was a circinate patch the size of a florin, having a somewhat intricate whorled pattern, suggesting tropical ringworm. On the outer side of the ankle just below the external malleolus there was a small patch the size of a sixpence, which was fading. No treatment had been applied to any of the lesions. There was no history of infection among his pupils, with the exception of one, a friend, who stated that he had some patches on his skin which he noticed while on the voyage from Japan. The patch on the groin was scraped, and in the scales thus obtained a very large-spored mycelium with unusually long branches was demonstrated.

(2) A case of *Pityriasis rosea* in a girl aged about 12 years, with very characteristic pale pink patches, in size about one quarter by half an inch, appearing first upon the upper part of the trunk above the clavicles, and arranged in lines directed obliquely from the summit of the shoulder to the clavicle. Upon the back in the intervertebral groove there were numerous similar patches and also smaller papules, round and faintly scaly. The eruption had made its appearance two days previously, and it was not now present upon the limbs or lower upon the body than the groin. It itched slightly. There was no similar eruption in any member of the family. This was the second case the exhibitor had seen of the disease this week, and it was probably true that the malady was an exanthem occurring in epidemic form during certain periods of the year.

Dr. WILFRID WARDE agreed as to the epidemic character of the disease, and he remarked upon the general good health of the patients. He

thought that the so-called “herald-patch” was frequently missed, both by the patient and the physician.

(3) A case of *Lupus vulgaris* of the nose and cheek in a little weakly East-end child who had had a remarkably complete series of tubercular affections, commencing with a tubercular ulcer upon the right conjunctiva and cornea for which the eye had been enucleated eight years ago. Six years afterwards she developed lupus of the nose, which had run a very acute course and had speedily ulcerated, producing considerable loss of tissue in the cartilaginous portion of the nose. She was treated at first with cod-liver oil and thyroid extract for some months, and later with X-rays, which had been continued for many months with apparently no good effect. After several months of this treatment sudden improvement took place up to a certain stage, but there it stopped, the nose being still the seat of active disease. The application of Finsen light by the Finsen-Reyn lamp was then tried, and, in all, sixty exposures had been given of an hour and a quarter at a sitting, at daily intervals. The result had been satisfactory beyond belief to those who had seen the case at its worst. It was also an interesting fact that this patient had been the subject of an acute attack of typical Lichen scrofulosorum of the trunk, and she had been shown with this eruption at the Dermatological Society of London, in the *Transactions* of which it was fully recorded.^[7]

All the members agreed upon the excellent results which had been obtained in the treatment of this case.

(4) A case of *Linear lichen planus* in a middle-aged woman, who had been under observation since August of last year. At that time she presented herself with no symptoms of the disease except a broad, linear patch running vertically across the popliteal space on the left side. This patch was of a violaceous tint, and the skin was thickened along its course and was intensely irritable. But there were no papules distinctive of *Lichen planus* on any part of the body, until about a month ago, when she began to develop an acute attack of the disease. It was interesting to observe that new papules continued the line of the original streak from the popliteal space up the back of the right thigh as far as the buttock in an almost unbroken line, other papules being distributed extensively upon the body, but without any

linear arrangement. She had well-marked lesions of Lichen planus upon the buccal mucous membrane, which had also developed within the last few weeks. The itching was a very severe symptom, and it kept the patient awake at night.

(5) A case of *syphilis* in a man, aged 25 years, with an affection of the upper lip which it was a little difficult to classify, the question being whether it was a primary or a tertiary ulcer. The whole of the lip was greatly swollen, and at each angle of the mouth there was a deeply-excavated ulceration scabbed over, the intervening portion of the lip being occupied by a papillomatous infiltration resembling the appearance of frambœsiform syphilis. The man absolutely denied previous lesions of any kind; there was no scar upon the penis or upon any portion of the body, and no history of a secondary rash at any time. But the uvula had been apparently destroyed by ulceration and was now merely rudimentary, and there was a deep ulcer upon the back of the pharynx very suggestive of tertiary disease. The absence of primary and secondary symptoms was peculiar. It should be mentioned that there was no pronounced enlargement of the glands in connection with the deep ulcers of the lip, which, it might be considered, precluded the diagnosis of primary sores here.

Mr. ARTHUR SHILLITOE suggested the possibility of the case being an instance of tertiary symptoms in a patient the subject of congenital syphilis, but the teeth and physiognomy generally did not confirm this possibility. The general opinion supported the diagnosis of tertiary syphilis.

Mr. GEORGE PERNET showed a case of *superficial scarring and telangiectases of the left side of the face and neck following an X-ray burn*. The patient, aged 28 years, was treated three years ago in the provinces for what was said to be a tuberculous lesion upon the left side of the chin. As far as she recollected, the affected part was exposed on eight occasions to the X-rays for about ten minutes at a time, the eyes only being protected, but not the adjacent parts of the face and neck. The patient stated that as a result the face was dressed and bandaged for three months. The parts now exhibited the well-known characteristic features following such burns. The original lesion for which she had been treated appeared to be healed.

Dr. V. H. RUTHERFORD showed microscopic sections from the case of *multiple sarcoma cutis* which he had exhibited at the previous meeting of the Society:

(1) An early nodule—the size of a pea—over which the skin still retained its natural colour, displayed chiefly fibrillary bundles with a small number of cells in various stages of development, some being round, others elongated with fine protoplasmic processes, and others again with irregular outlines.

(2) In a later nodule—the size of a bean—over which the skin had become red, the bundles of fibres were replaced by typical small round cells (lympho-sarcoma). The sections were stained with hæmatoxylin.

A post-mortem examination was made on Monday, February 20th, when a considerable growth was found in the mediastinum (probably primary), pressing upon the right bronchus, and smaller growths sparsely distributed in the pleura, lungs, retroperitoneal glands, etc.

CURRENT LITERATURE.

ON THE QUESTION OF BLASTOMYCOSIS OF THE SKIN AND ITS RELATION TO "FOLLICULITIS EXULCERANS SERPIGINOSA NASI" (KAPOSI.) BRANDWEINER. (*Archiv f. Derm. u. Syph.*, August, 1904, p. 49. One plate.)

This paper is based on a case which occurred in Dr. Matzenauer's clinique in Vienna. The patient was a shoemaker, aged 37 years, who suffered from a papillated crusted lesion on the lower part of the left side of the nose. The diseased patch occupied the ala nasi, and extended up as far as the bridge, where it faded into the surrounding skin. It was reddish-brown in colour, raised and irregular in shape, and covered with yellowish inspissated pus. It had been noticed first about a year before. On histological examination the stratum corneum was found to contain numerous unicellular organisms of from 4 to 10 μ in diameter, which stained with polychrome methylene blue, were doubly contoured and were believed to be blastomycetes. The epithelium had proliferated and presented small necrotic pustules containing these organisms. There was a round-celled infiltration in the corium in which the blastomyces was also detected. An attempt to cultivate the organisms and to inoculate them in lower animals was unsuccessful. On the ground of this not quite convincing case the writer took the opportunity of reviewing the literature on the subject. He referred in detail to several cases described by Kaposi with the title of "Folliculitis exulcerans serpiginosa nasi," which he regarded as most probably identical with "blastomycosis."

J. M. H. M.

A NOTE ON THE TREATMENT OF SYPHILIS. JONATHAN HUTCHINSON. (*The Practit.*, August, 1904, p. 145.)

In this short communication the writer refers at the outset to the remarkable fact that mercury obtained its reputation in the treatment of syphilis almost immediately after the introduction of that disease into Europe, and “the pills—which were known as Barbarossa’s, and of which a large quantity was furnished for the treatment of Francis the First—were probably essentially the same as those which Messrs. Burroughs Wellcome and Co. now supply by the million.” In opposition to a number of writers, especially on the Continent, the writer is a strong advocate of the early use of mercury, and of what has been called the “suppression treatment.” He commences mercurial treatment immediately the syphilitic character of the sore is definite. The prevention of late or tertiary phenomena being the most important aim in the treatment of the disease, efforts should be made to this end, and he considers that the immediate use of mercury, which may prevent the secondary stage from developing, has also the effect of reducing the possibilities of the development of the late stage. With regard to the length of time the patient should continue to take the mercury the writer strongly believes in the efficacy of prolonged and continuous treatment. He does not see the object of intermitting the course and giving the specific virus any chance of renewing its activity unless the mercury is interfering with the health of the patient. The plan of treatment usually adopted by the writer is as follows: “A pill is prescribed containing one grain of grey powder and one of Dover’s powder, and this the patient is to take after meals, and only three times a day at first. If no diarrhoea follows, the pill is to be given four, five, or six times a day.” The diet is regulated, and the patient is told that the treatment must be continued without intermission for a year at least. An alum mouth-wash is ordered to prevent ptyalism. If there is debility, a grain of quinine is added to the pill. In using the iodides he considers that the iodides of mercury are much less manageable than the two separated, and prefers to give the iodide of potassium in a fluid dose along with the mercurial pills.

J. M. H. M.

URANIUM IN THE TREATMENT OF LUPUS: A PRELIMINARY NOTE. NORMAN WALKER. (*The Scot. Med. and Surg. Journ.*, September, 1904, p. 207.)

The method of applying the uranium which was adopted by the writer was to prepare an oxide of uranium from the nitrate, incorporate it with a negative base such as bees-wax, and spread this on leather to form a plaster of the usual thickness and the required extent. The plaster is enclosed in waxed paper gummed at the edges. The uranium plaster was applied at first in cases where the limbs were affected with lupus and later on the face. Exposure for three days and three nights to an affected arm was followed by no reaction, but on the face it was found that a slight reaction followed in cases where the exposure exceeded twenty-four hours. The plaster may be worn only at nights and removed in the morning. The results have been encouraging, and the risks appear to be negligible. The treatment is cheap, since the cost of making the plaster has been estimated at about one shilling, and experiments have shown that the plaster remains active after four months. This short paper is illustrated by reproductions of four skiagraphs, one taken by thorium and the other three by uranium plasters. These demonstrate the fact that the radio-activity of the uranium salt is much greater than that of thorium.

J. M. H. M.

OBSERVATIONS ON THE USE OF EUCAINE B. AND ADRENALIN AS A MEANS OF INDUCING LOCAL ANÆSTHESIA. GEORGE L. CHIENE. (*The Scot. Med. and Surg. Journ.*, September, 1904, p. 215.)

In this communication the writer makes some interesting and instructive observations on the value of combining adrenalin with encaine or cocaine as a local anæsthetic for minor operations. Satisfactory results were obtained by employing a mixture of a 2 per cent. solution of eucaine B. and 1-5000 adrenalin chloride, thirty minims of the solution being injected. The chief drawback to the method was the fact that adrenalin solutions did not keep well, and readily became inert and contaminated. This difficulty has now been obviated to a large extent by the introduction by Messrs. Burroughs Wellcome and Co. of a new preparation of the supra-renal gland called "Soloid Hemisine," which is said to represent the hæmostatic principle of the supra-renal gland in a more stable form. The writer has had compound soloids prepared by the same firm which when dissolved in 10

c.c. give a solution of 1 per cent. eucaïne B. and 5 minim. of adrenalin (1-1000.) By injecting this a carbuncle the size of the palm of the hand was excised without pain or discomfort, and at a later date the surface was skin-grafted, the same anæsthetic being used. The writer believes that the addition of the adrenalin not only increases the efficiency but also the safety of the eucaïne or cocaine as local anæsthetics.

J. M. H. M.

DERMATITIS FRAMBÆSIFORMIS. F. SAUERBERGER (*Archiv f. Derm. u. Syph.*, October, 1904, lxxii, p. 3).

In this contribution the writer describes an unusual case which occurred in Professor Janovsky's clinic at Prague. The patient was a coal-miner, aged 18 years. At the age of 14 he began work in a mine and soon afterwards he suffered from a severe attack of furunculosis, which persisted for a year. As it got steadily worse he had to leave his work for a time. When he was able to work again he got employment in another mine. After some time he went to the original mine and the furunculosis returned rapidly and he was again compelled to leave work. He blamed bad drinking-water as the cause of it. Several other miners in the same pit were similarly affected and among them was the patient's father. Soon after leaving work for the second time the pustular lesions began to be replaced by raised papillomatous masses which were especially well-marked about the face and neck. The eyelids, forehead, tip of the nose, nasal orifice, lips, chin, and left side of the cheek became covered with crusted papillomatous lesions. A few similar lesions developed on the body in the inguinal regions, and both the cervical and inguinal glands were enlarged. A microscopical examination of one of the lesions showed a proliferation of the interpapillary processes, and a dense cellular infiltration in the papillary and sub-papillary layers, consisting chiefly of leucocytes and mast-cells and dilatation of the capillaries. The lesions did not react well to local treatment, but eventually with tonics and by persevering with local remedies a cure resulted. There was no history or evidence of syphilis. The disease was similar in many respects to tropical frambæsia or yaws, and corresponded closely to a case which Lewin described as sporadic frambæsia.

J. M. H. M.

ON THE RADIO-THERAPEUTICS OF RINGWORM. SABOURAUD.
(*Ann. de Derm. et de Syph.*, July, 1904, p. 577.)

In a previous paper (published in January, 1904, in the *Annales de l'Institut Pasteur*) Sabouraud had proposed a method of treating ringworm with X-rays, which he sums up as follows: "The application in one sitting upon a given point of the scalp of a quantity of X-rays equal to 4-1/2-5 H-units of Holz knecht is made; fifteen days later a complete epilation will occur on the region treated, healthy as well as diseased hairs being shed. New and healthy hair will commence to grow ten weeks after the treatment, and will be completely restored within ten weeks. The infectivity of the disease will cease with the fall of the last diseased hair, within twenty-five days at latest of the treatment." This formula, Sabouraud maintains, has been amply proved to be true by his experience of the six months which have intervened since his paper of January, 1904. Some very striking results are quoted. Thus while in six months before this treatment was adopted 57 patients were discharged as cured, in the past six months 134 such patients were discharged; moreover, a very large number of patients were treated without admission and also cured. An entire section of the hospital, consisting of one hundred beds set apart for favus cases, has been closed and will be converted to the uses of general medicine, as being no longer required for its former purpose.

Certain additional knowledge has been afforded by the experience of the past six months, which may be stated in the following practical rules: The penetration of X-rays is proportional to their number; the more penetrating they are, the more numerous they are. The danger of X-rays is proportional to their penetration; it is thus more dangerous to handle X-rays of 8°-11° (on Benoist's radio-chromometer) than rays of 3°-5°. Now tubes change progressively with use and these changes are marked by colour changes in the glass. At first, after about ten hours' use, the tubes take a violet coloration; with this there is no impairment of function. But with further use, after about thirty hours, a brown tint becomes marked, and the tube is said to be "smoked." With this change it becomes increasingly difficult to obtain rays of a low degree of penetration. For epilation rays of all degrees of penetration serve equally well; rays of 3° are as effective as rays of 11°, but the treatment must be for a longer time. Thus a tube working at 3° caused an epilation in twenty-five minutes. The same epilation would result

in eleven minutes from a tube at 8°-11°; and if one used the latter tube for twenty-five minutes, a dermatitis would result. This is entirely unnecessary for epilation and is attended by mischievous consequences, even when quite slight, and must be avoided. All forms of dermatitis, except that of transient erythema, produce permanent baldness. Permanent alopecia may also result without dermatitis in cases where the amount of X-rays given has been insufficient to cause epilation, and a second *séance* has been necessary. In this case certain hairs will have received a dose beyond the maximum for safety, and these will be permanently destroyed. For safe working, it is necessary to have a measure of the rays coming from the tube at any given time; and the measure must be constant for every variety of tube. This is obtained by certain colour changes taking place in specially prepared substances, these changes being proportional to the amount of X-rays derived from the tube. The pastilles of Holz knecht have been used for this purpose, but they are a secret preparation, expensive, and not everywhere procurable. Sabouraud has devised small discs made of paper which is the same as that used for screens and is coated with a preparation of platino-cyanide of barium. These discs change in colour in a fixed manner, comparable from time to time with test-colours, and their use makes it impossible, with care, to overstep the limits of safety.

E. G. L.

SYPHILIS AND CANCER. ETCHEVERRY. (*Ann. de Derm. et de Syph.*, August-September, 1904, p. 797.)

This research was undertaken in Audry's Clinique at Toulouse, and the consideration of the connection between these two diseases was directed to the occurrence of lesions on the tongue and buccal mucous membrane. These facts are grouped in three classes, according as: (1) cancer supervened, in syphilitic patients, upon a preceding leucoplakia; (2) without leucoplakia, but in the presence of other local syphilitic lesions, such as gummata, scars of gummata, or of hard chancres; and (3) in the absence of all local signs of syphilis. Thirteen cases of epithelioma developing upon syphilitic leucoplakia are detailed under the first heading. This sequel is relatively frequent. In the second group twenty-two cases are noted. In the third group no direct transformation of syphilitic into epitheliomatous

lesions could be established, but the fact of previous syphilis was ascertained in all the fifteen cases quoted. Etcheverry, struck with the singular frequency of association of the two diseases, speculates whether there may not be some direct predisposition in syphilitic patients, as such, to develop cancer. In epithelioma of relatively young patients he states that syphilis may almost always be found as a previous accident. The greater frequency of cancer of the mouth in men than in women coincides with the greater frequency of syphilis in men; according to Fournier, syphilis is eight times more common in males. Etcheverry hazards the suggestion that the cachexia produced by syphilis may be a directly provocative cause of ensuing cancer. As regards the treatment of these mixed cases of syphilis and cancer, the administration of mercury, preferably by injections of calomel, influences favourably the syphilitic element of the disease, and occasionally, as a temporary effort, the epitheliomatous development is checked by this drug. On the other hand, iodides are distinctly mischievous to the malignant growth, and should not be given.

It is probable that the same close connections obtain between cancer and syphilis of the rectum as has been here noted in the case of the mouth, but observations on this point are too few to be of value.

A very full bibliography is added to this careful paper.

E. G. L.

ON ADENOMA SEBACEUM. BUSCHKE. (*Derm. Zeitschr.*, Bd. xi., Heft 7, p. 467.)

The patient was a boy, aged 13 years, in whom the eruption of small tumours had occurred at the age of five, coming out suddenly after an attack of measles. The distribution and appearance of the single lesions on the face was of the characteristic type of reddish-brown nodules situated on the nose and contiguous portions of the cheeks. He had also numerous fibromata on the back, and a widespread eruption of small, pale yellow, seed-like tumours all over the back, the little tumours resembling milium, and on close examination appearing to be pierced by lanugo hairs. A histological examination of all classes of tumour showed them to be either pure fibromata or angio-fibromata, though Buschke admits that in the case of the

face tumours the excision was so superficial that it is possible that deep sebaceous glands might be present below the point at which the excision took place. The boy was clever at work, and generally showed no physical or mental defect other than the presence of the cutaneous anomaly.

A. W.

ON THE PATHOGENESIS OF COLLIQUATIVE BULLÆ. KREIBICH.
(*Derm. Zeitschr.*, Bd. xi, Heft 5, p. 315.)

After reviewing shortly the theories at present put forward for the production of colliquative bullæ, Kreibich enters into great detail in the description of his experiments. He finds evidence that in *Zoster hystericus* there is an acute œdema consisting of fibrinous exudation which enters into the cells and causes their necrosis without separating them from their deep attachments. By irritating the same part three times running with the poison of *urtica urens* Kreibich was able to produce a colliquative vesicle, since the œdematous process then reached to the papillary body. Similar is the pathogenesis of true herpes zoster and in all probability of hydroa vacciniforme. To sum up the results of the investigation he states that "Colliquative bullæ are found in processes which are allied to urticaria but differ from this in the greater duration of the vascular lesion and by its situation in the papillary body. The colliquation of the epithelium is produced by the action of an exudation rich in fibrin on the epithelial cells, which have suffered in their nutrition but which have maintained their connection with the cutis. The disturbance of nutrition is a combined result of the vascular lesion, the pressure of the exudation, and the resultant anæmia."

A. W.

FORMALIN AND ITS ACTION ON FAVUS MOULD. BOGROW AND
SCHARKEWITSCH-SCHARSCHINSK. (*Derm. Zeitschr.*, Bd. xi, Heft 5, p. 329.)

This is the record of careful investigations of the action of formalin on the achorion, both culturally and on the diseased scalp. The results show the absolute futility of using formalin in a watery solution, but the authors think

that further work on the action of spirituous solutions and vapour is desirable.

A. W.

ON A CASE OF ERYTHEMA NODOSUM OF UNUSUAL COURSE
AND ATYPICAL LOCALISATION. EINIS. (*Derm. Zeitschr.*, Bd. xi,
Heft 7. p. 493.)

A little boy, aged 2-1/2 years, was brought up with four slightly raised swellings, two, each as large as a half-crown, being situated symmetrically on the cheeks, and two, each as large as a sixpence, on the temples.

The history showed that exactly a year before the child had suffered from a similar eruption on the lower legs, and the doctor who treated the child for former attacks identified the eruption with the fresh one. The peculiarity in the course showed itself in the access of colour and pain in the nodes every day about noon, though there was no fever, and the child seemed to be in very good health with the exception of a few swollen sub-maxillary glands.

A. W.

SEVERAL CASES OF ATROPHY OF THE SKIN. ALEXANDER. (*Derm. Zeitschr.*, Bd. xi, Heft 5, p. 338.)

Four cases in all are alluded to in this paper. The first patient, a man aged 31 years, showed all the symptoms of an epidermolysis bullosa, with slight ulceration, but the affection was said to have begun at the age of 18 and only to occur periodically. The second case was that of an old woman, aged 70 years, who also had an abdominal tumour, of which she died. Material showed that there was a complete loss of the papillæ, the elastic tissue had disappeared to only the slightest traces, and there were signs of inflammatory infiltration present. The third case was that of a woman, aged 55 years, with marked syringo-myelia; and the rash, which appeared as small nodules, terminating in atrophy, affected the axillæ, the inner sides of the upper arms, the groins, the inner sides of both thighs, the back, and abdomen. The author says that one is tempted to connect this with the condition of the cord, but to do so is the purest theoretical speculation. The

fourth case was a woman, aged 42 years, who showed on the left side of the nose a circular patch as large as a sixpence, over which the skin was of a bluish-red colour, and somewhat infiltrated. There were some scars caused by the therapeutic use of electrolysis.

(From the short description of this last case it does not seem clear why the diagnosis of Lupus erythematosus was not made.—A. W.)

A. W.

ON THE TREATMENT OF TYLOSIS PALMARIS IN ADULTS.
MAYER. (*Derm. Zeitschr.*, Bd. xi, Heft 5, p. 865.)

Mayer recommends the rubbing in of rheumasan, a salve-soap containing 10 per cent. of free salicylic acid. This should be massaged in for at least five minutes and then glacé gloves should be worn at night. The patient should be cautioned to discontinue the use of the drug for a few days on the first appearance of redness.

A. W.

PSORIASIS VULGARIS OF THE SKIN AND MUCOUS
MEMBRANE, ITS PATHOLOGICAL POSITION AND ÆTIOLOGY.
P. THIMM. (*Monatsh. f. prakt. Derm.*, July 1st, 1904, p. 1.)

Dr. Thimm gives the history of a patient, a man aged 36 years, who had been under his care for five years. He had an obstinate recurring extensive psoriasis eruption of the skin of the trunk and limbs, typical psoriasis lesions of the upper and lower lips, extending on to the mucous membranes of the mouth and nasal cavities. In addition, the mucous membrane of the mouth showed, while he was under observation, various circumscribed diseased patches, setting in synchronously with the eruption on the skin, which also disappeared at the same time after general treatment with arsenic. Schimmer's typical leucoplakia was also present, the patient being an immoderate smoker, and this continued in spite of all treatment. There were no signs of syphilis.

Microscopical examination of two mucous membrane lesions showed great œdema, enlarged intercellular spaces, thickening of the epidermal covering *in toto*, but a thinning of the rete over the elongated, high-reaching papillæ. Small-celled infiltration of the upper layers of the corium, extending even up into the epithelium, was also present.

But what was not visible, nor present in any of the sections, was the parakeratotic heaping up of scales of the psoriasis lesions. Exceptionally this may be wanting in cutaneous psoriasis patches, but its absence is, as a rule, or even always, characteristic, in psoriasis of mucous membranes. In certain sections, however, the uppermost epithelial layers, one found, to a depth of three cell layers, were pressed together to form a dark-coloured narrow band, but having regard to the smooth contour of the surface this could hardly be looked upon as a partly developed scale. The pathological characters of any lesions are to a certain extent altered according to the position in which they occur. By the word “parakeratosis” is understood, apart from the heaping up of dry, scaly formations, a condition in which we find the following changes of the epithelium: parenchymatous œdema, extension of the basal horny layer, abnormally scanty fat contents of the same, want of keratohyalin, and abnormally good preservation of the nuclei. Such are, however, pathological changes of the external epithelium, but normal conditions of the mucous membranes. Parakeratotic changes of the mucous membranes of the mouth cannot be considered diagnostic of psoriasis. But the changes in the papillæ, the infiltration of the upper layer of the corium, the widening of the blood-vessels, the dilatation of the lymph-channels, taken in conjunction with the clinical symptoms and the results of treatment, leave no doubt that the case was one of true psoriasis vulgaris of the mucous membrane of the mouth. Previously, very few cases of scaly lesions of the mucous membranes had been described, and in psoriasis of the skin the extensive and destructive infiltration seen in these lesions of mucous membranes is unknown.

The ætiology of psoriasis, whether of the skin or mucous membrane, has been, and is still so hotly discussed, that the translator can but mention the author’s conclusion that the pathological picture of his case points anatomically to an inflammatory origin, and ætiologically to a parasitic origin of psoriasis.

J. L. B.

ERYSIPELAS NEONATORUM GANGRÆNOSUM. E. NOHL.
(*Münch. med. Wochenschr.*, September 13th, 1904, p. 1648.)

The child was born on February 29th, and was apparently healthy at birth; she developed diarrhœa on the fourth day, and redness and swelling of the genital region on the sixth.

Seen for the first time on March 7th, the labia majora were much swollen, red and shining and painful. Both thighs presented a firm œdema and looked blue and marbled. There were frequent motions like pea-soup. On March 9th the temperature rose above normal for the first time and, with the exception of the first two evenings, when it sank below normal, it remained elevated till the child died, reaching 40° C. on March 11th, and 40·5° C. on March 15th.

On March 12th a large ulcer appeared on each labium following a bullous upheaval and casting off of the epidermis, and at the same time the skin over the trochanteric region on each side commenced to necrose. The œdema had involved the whole of the lower extremities, the affected parts being pale and flecked with blue; small bladders filled with yellow serum appeared on the dorsal surface of each foot.

Fresh areas of necrosis developed. The ulcers on the labia turned black. On March 15th the child died, with symptoms of meningitis. A post-mortem examination was not allowed.

The author discusses the differential diagnosis between erysipelas and œdema neonatorum. He quotes a passage from Strümpell, as well as a case of his own, to show that fever sometimes appears after the eruption in erysipelas. He further records the fact that there had been a great deal of infectious illness in the house before the birth of this child—severe measles, whooping-cough complicated by empyema, ulcerated throats with otitis media—whilst the mother developed parametritis on the second day.

W. B. W.

1. *Amer. Journ. Cut. Dis.*, vol. xxi, 1903, p. 315.
2. *Vide* X. Delore, *Lyon Médicale*, July 16th, 1899, p. 376; J. Sabrazès and A. Laubée, *Arch. Gén. de Méd.*, Paris, November, 1899, p. 515; R. von Baracz, *Wien. klin. Wochenschr.*, 1901, No. 14; G. Carrière and G. Potel, *Presse Médicale*, Paris, May 17th, 1902, p. 471; and L. Legroux, "La Botryomycose," *Thèse de Paris*, 1904. The botryomyces appears to have owed its supposed existence to a mistaken interpretation of microscopic appearances. H. Bichat (*Arch. Gén. de Méd.*, February 2nd, 1904, p. 281) thinks there is nothing specific in the growths, but V. Ball (*Arch. Gén. de Méd.*, August 2nd, 1904, p. 1921) concludes that that botryomycosis, though it owes its name to an error, is nevertheless a pathological entity and is a special staphylococcal affection. Dr. Weber possesses a microscopic section of one of these little growths, which was removed from the finger (close to the nail) of a woman in 1890, when he was a house-surgeon at St. Bartholomew's Hospital for Sir William Savory. It was a typical strawberry-like "botryomycoma" with quite a narrow pedicle. Dr. J. M. H. MacLeod, who has kindly examined the section in question, tells us he regards such growths as "septic granulomata," septic organisms producing very various effects according to their degree of virulence and naturally according to the nature of the living soil on which they grow.
3. *British Journal of Dermatology*, June, 1901, p. 201.
4. *Diseases of the Skin*, third edition, 1903, p. 963.
5. *British Medical Journal*, June 4th, 1890. Amongst foreign accounts *vide* Radaeli's report of five cases in *Lo Sperimentale*, December, 1904, p. 1023.
6. *Archives of Surgery*, vol. v, p. 237, and vol. vi. p. 132. See also Mr. Hutchinson's *Smaller Atlas of Clinical Surgery*, Plate 61, where he refers to the case figured by Hebra and Kaposi as "Sarcoma melanodes"; also his remarks on "Sarcoma melanodes" in the *British Medical Journal*, January 7th, 1905, and the case shown by him at the

Polyclinic in December, 1904, which was also shown by Dr. Ormerod at the Dermatological Society of London, July 8th, 1903.

[7.](#) *British Journal of Dermatology*, 1903, p. 210.

Transcriber's Notes:

Missing or obscured punctuation was corrected.

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